Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0017	**Age / Sex**	45 / F
Setting	Emergency Dept	**Primary dx**	Carbon monoxide poisoning

Chief complaint

Headache, vomiting, then collapse at home.

History of present illness

Family of 3 had headache and nausea for 2 days during cold weather; she was found drowsy in a bedroom next to a malfunctioning gas boiler. Husband and son also symptomatic but alert. Fire service detected elevated CO levels.

Past history / medications

Asthma, salbutamol inhaler. Non-smoker. Not pregnant.

Examination

HR 118, BP 108/64, RR 22, SpO2 99% on pulse oximetry (unreliable). Cherry-red skin not evident. Mild lateral nystagmus, no focal weakness. Pupils equal, reactive.

Glasgow Coma Scale

Eyes open only to pain. Confused, speaks in short disoriented phrases. Motor: localizes painful stimuli.

GCS on arrival	Score	Response
Eye opening (E)	2	To pain
Verbal (V)	4	4 - Confused
Motor (M)	5	Localizes pain
TOTAL GCS		**11**

GCS after hyperbaric oxygen	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**15**

Investigations

Co-oximetry: carboxyhemoglobin 34%. Lactate 4.4, pH 7.29. Troponin mildly elevated (110 ng/L), ECG sinus tachycardia with ST depression V4-V6. CT head: no acute abnormality.

Assessment

Severe carbon monoxide poisoning with neurological and myocardial involvement. GCS 11.

Plan / disposition

High-flow 100% oxygen via non-rebreather mask, then hyperbaric oxygen therapy given loss of consciousness and cardiac injury. GCS 15 after treatment. Whole household evaluated, gas boiler removed from service. Cardiology and neurocognitive follow-up arranged.